DR Care Solutions

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Overview for Residential Aged Care Placement

CLIENTS NAME: **JELENA 'HELEN' SRDIC**
DOB: **10.09.1947 (77 years of age)**
CLIENT ID: **DRCS 0547**

This overview is a summary completed for all clients requiring Placement into Residential Respite or Residential permanent aged care, assisted living or group home. The overview is undertaken by a consultant from DR Care Solutions. It's used as a guideline outlining the background and needs of a specific client(s).

Issues covered:	**Background and relevant history;** Joseph Portelli and Kate Cramsie from Lorica Partners reached out to Danielle from DR Care Solutions on 8th April 2025 to discuss their client, Helen Srdic who has been very unwell in Hospital since October 2024. They made an e-introduction to Helen's two children, George Srdic and Biljana Veledinovic in relation to having support in relocating Helen out of hospital and into a supported living environment in their desired location. George and Biljana agreed to meet with Danielle via Zoom on Monday 14th April 2025 and again on Wednesday 16th April 2025 to provide information for this Overview for Placement. **Past medical history:** Helen was diagnosed with Glioblastoma multiforme of the brain (Brain Cancer) in June / July 2024 after a stroke type episode. Helen underwent scans and was referred to a neurosurgeon. She underwent surgery on 2nd August 2024 and returned home during her recovery period. She underwent intensive chemotherapy and radiation. Helen developed seizures and has been in and out of Westmead Hospital and Westmead Rehabilitation Hospital since October 2024. She currently remains in Westmead Public Hospital, Oncology Ward, whilst awaiting placement. She is unable to return home due to her very high care needs. Other medical issues include high cholesterol, hypertension, deep vein thrombosis (DVT) of her lower limbs, macular degeneration, thyroid nodule, mobility issues and dysarthria (slurred / indistinct speech).
• Physical health including mobility, continence, • Cognitive health • Sensory (sight, hearing, oral, feet) requirements • Activities of daily living • Medications, allergies & vaccinations • External assistance - including Allied Health • Emotional & mental health • Social background, networks and Relationships • Likes & dislikes	

● Cultural linguistic and Spiritual needs ● Estate Plan/ Advice and Life Ready Register ● Funeral arrangements ● Financial Plan/ Advice ● My Aged Care ● Criteria for next home	**Physical health including mobility, continence:** Helen is currently a two person assist for all transfers and mobility including into a tilt in space wheelchair. She requires pressure area management and regular repositioning and massage. She's virtually bed-bound in hospital. Staff resourcing in the hospital system has been an issue to get her up and into a chair. She hasn't been outside since October 2024. No further physiotherapy sessions have taken place, and she has developed significant muscle wastage. Helen's family would like to see some *one-on-one* physiotherapy to ensure she's getting the best chance possible of mobilising and maintaining muscle mass. Helen is fully incontinent with an indwelling catheter in place to manage her urine. She has developed a C. Diff infection of her bowels which is being resolved. She remains incontinent of bowels as well. **Cognitive health:** Helen remains with severe swelling on the brain and short-term memory issues. Her long-term memory is intact. She has word finding difficulties and needs patience from those communicating with her. **Sensory:** Helen wears glasses for reading and watching television. She has Macular degeneration (wet type) requiring injections on a six weekly rotation basis however she hasn't had them given she's been in hospital. Her vision isn't blurred. Helen's hearing is intact. She doesn't have nor need hearing aids. She has all her own teeth. Helen needs a professional podiatrist to maintain her foot health and nail care. **Activities of daily living:** All of Helen's activities of daily living are undertaken by two carers. She is bathed in bed. Helen is managing a soft diet and needs her food set up and assistance with feeding. Her appetite remains good. Helen is easily fatigued and sleeps during the day. Helen used to sleep extremely poorly prior to her diagnosis. **Previous standard day**: Helen used to wake around 6.30 – 7am. She was a keen walker and often walked with her neighbour, made her own breakfast, tidied the home, undertook the washing and was working from home and in the office before her diagnosis. She would work until late and often fall asleep at the computer around 10.30-11pm. She slept poorly since losing her husband 12 years ago. She'd prefer to sleep on the lounge rather than in her bedroom. She was previously very independent, active and supportive of others. She constantly had the radio on either talk back radio or music of her era (2UE or 2CH). **Emotional & mental health:** Helen's current mental health isn't good. She's very down and a noted decline in her mood. The hospital is arranging her to see a psychologist or psychiatrist to provide some support either with counselling or possible stronger anti-depressants.

Medications, vaccinations & allergies: We have received a current list of medications from the hospital however that will need to be updated prior to admission into care. We understand Helen is allergic to ***codeine.*** She is fully vaccinated.

Current medications include:

- Brivaracetam (anti-convulsant medication)
- Cefalexin (antibiotic)
- Clonazepam (anti-convulsant medication)
- Dexamethasone (corticosteroid to reduce inflammation)
- Digoxin (to treat heart conditions)
- Docusate senna (laxative, stool softener)
- Enoxaparin (anti-coagulant to treat blood clots, DVT's)
- Melatonin (natural supplement to assist with sleep)
- Metoprolol (to treat hypertension)
- Midazolam (to assist to sleep and relax, sedative)
- Oxycodone (opioid analgesic for moderate to severe pain)
- Pantoprazole (to treat stomach acid, reflux, ulcers)
- Paracetamol (to treat pain)
- Rosuvastatin (to treat high cholesterol)
- Sertraline (an antidepressant)
- Silver Sulfadiazine topical (antibiotic medication to treat infections by bacteria)
- Trimethoprim-sulfamethoxazole (combination antibiotic to treat various infections)
- Vancomycin (antibiotic to treat serious and resistant infections)

Social background, networks, and relationships: Helen was born in Passau, West Germany. She had one brother Michael who passed away 2 years ago. Helen came to Australia at the age of 2 years and settled with her family in Sydney. She attended primary school and high school in Sydney. She undertook a secretarial college course after leaving school. She did secretarial work for a company in Broadway before obtaining a job at Cottee's in Liverpool. She was a personal assistant to several managers there. She worked for over 20 years with Cottee's before being made redundant.

Helen found a role and worked in export administration up until her diagnosis in mid-2024 (at 77 years of age).

Helen met her future husband, Danny in 1967 and married him in 1969. George was born in 1970 and Biljana was born in 1973. Danny passed away in 2014. She's been living alone in her single level home in Smithfield with her dog since that time. She only had one year of maternity leave after each birth and worked continuously.

	George has three children: Emilie (17), Alexander (15) and Isabel (12). Biljana has two children: Marko (21) and Aleksija (15). Both George and Biljana work full time and have busy lives. They've been visiting Helen every day and split the times they visit to give Helen extended visits from family. It's been tiring and challenging for all however they are a very closeknit family with strong family values. Helen has very close family friends who are like family (Cumpari). They would spend quality time each Saturday and cook together. It was a great honour for them to spend time with each other. Some of Helen's school friends have kept in touch and have visited her while in hospital. Several people from her work have also kept in touch with her. **Likes & dislikes:** Helen used to love to cook and was a good cook. She previously had her gall bladder removed therefore she doesn't eat deep fried foods. She dislikes spicy fees or curries. Her preference is for a Mediterranean diet with a strong protein focus. She's currently having Ensure Plus to help her protein intake. Her doctors have recommended she focus on a Ketogenic diet / alkaline diet. She enjoys Italian style foods, meats, eggs and seafood. She enjoys tasty foods. George and Biljana advised she likes potato chips and sweets. Helen used to enjoy reading Barbara Taylor Bradford, Danielle Steele and romance novels. She loves watching Doris Day and Cary Grant and similar movies from her era, Lassie and romantic comedies. Helen continues to enjoy listening to music from the 1950's and 60's and has the radio on constantly. Helen was hoping to pursue gardening in her retirement. She loves roses and the family had a vegetable patch which she was keen to help nurture. Helen is very selfless and worries more about others than herself. She's a giver. She used to have boundless energy and resilience. She's stoic and has a heart of gold. Helen has a wonderful sense of humour with an infectious giggle. Helen can be quite social once she gets to know people however is nervous when meeting new people. She's humble and genuinely cares for others. She's extremely polite and has never done anything wrong. Helen loved to undertake retail therapy. She enjoyed the art of physically going shopping for clothes and other nice things from David Jones, Country Road, Peter Sheppard Shoes, Oroton handbags and Chanel. When asked what brings Helen joy, it's her children and grandchildren.

	Helen has never smoked nor drunk alcohol. Helen doesn't like brashness, flashiness or loud people. **Cultural, linguistic & Spiritual needs:** Helen is Serbian orthodox. She can speak fluent Serbian and English with a bit of Russian. She loves her traditions and beliefs and focuses her religion on Easter, Saints Day and Christmas. She doesn't attend church however is devout in her beliefs. **Estate Plan/ Advice:** We understand that George and Biljana are joint Powers of Attorney and Enduring Guardians for Helen. We've requested e-copies of these documents from their financial adviser, Kate Cramsie who in turn has requested copies from Helen's lawyers. We will require copies of these documents prior to admission into care. **Advanced Care Directive**: Below we include a special DR Care Solutions link to Touchstone Life Care for building an Advanced Care Directive. You are welcome to use this or ask your GP for a paper-based document if it's preferred. This is usually stored in the client's GP Health Summary and hospital records should something occur, the medical profession adheres to the requests if the person is unable to communicate their wants, needs and wishes. Touchstone Life Care - A DR Care Solutions Partner **Funeral arrangements:** This was not discussed with George and Biljana although we recommend that it's a topic that should be discussed. The home the family selects will be requesting this information. We suggest that if you are unsure of who to select as a Funeral provider, we work with several providers in the Family Business sector who have obtained wonderful feedback on the service provision. We can make a warm introduction (if requested). **Life Ready Register:** For DR Care Solutions clients, we provide a copy of the Life Ready Register, a depository to keep all your affairs in one place. We suggest that you download this and start completing it. No matter what age, everyone can commence building this out and sharing it with their trusted network, their Powers of Attorney and Enduring Guardians as well as their Executor of their Will. Life Ready Register™ **Financial Plan / Advice**: Helen is a self-funded retiree. She was working up until the time of her diagnosis in mid-2024. She will be RAD (Refundable Accommodation Deposit) payer. We strongly suggest that the family discuss payment options and strategies with their own financial advisers, Kate and Joseph and reach out to Danielle if they need to speak with an Aged Care Accredited Financial Adviser who specialises in Aged Care funding options.

	My Aged Care: We have received Helen's ACAT support plan and referral codes for residential respite care and residential permanent care. Her assessment was undertaken on 6th March 2025. Referral codes herein: **Residential respite care: 2 – 158 481 289 585** **Residential permanent care: 2 – 158 481 289 530** **Criteria for next home:** *Location:* The family seek a home situated between Concord (where George lives) and Hinchinbrook (where Biljana lives) with a strong emphasis on good quality care from now until end of life. Somewhere midway between both children would be ideal. *Environment:* A single room which must be homey, preferably something where Helen can see outside. Accessible gardens are necessary as she wants to be able to go outside. *Medical capability*: they need to have a strong medical capability. Helen is gravely ill and has been battling infections. They request staff that are attentive and skilled and to be able to provide the care and attention she requires. *Staff culture* – they seek a place where staff that are genuinely kind and care. *Companionship* – Biljana and George spend 3 to 4 hours per day with her. They ideally would like to engage a private companion who can spend a few hours a day with her in a highly engaged way, ie. not scrolling on their phone whilst they're with her. NB: Danielle can arrange this as most Residential Aged Care homes don't have access to private carers one on one. *Allied Health Care*: Either on-site or contracted to include Physiotherapy, Occupational therapy, Speech pathology, Psychology, Podiatry and a Dietician.
Actions required (for the client to achieve these goals)	Danielle from DR Care Solutions is to locate a minimum of FOUR Aged Care homes between Concord and Hinchinbrook who can manage Helen's care needs now and into the future. They will need to be able to manage all her care requirements (outlined herein), have the availability for respite with a view to permanent care and be able to provide top quality care, support services with appropriate social interaction and activities for her to participate in within her physical abilities. **Her care needs are very high**.

	Once these have been located, pre-vetting, due diligence, clinical acceptance, and tours will begin. Helen must be clinically accepted into a home before we tour. We have diarised **Wednesday 23rd April 2025 as a possible date for tours** of local homes with a view to relocate Helen as quickly as possible after the tours. It will come down to locating a suitable home with an available room with clinical acceptance of Helen.
Who will be involved: (Guardian, solicitor, financial planner / advisers, family members)	• George Srdic and Biljana Veledinovic • Danielle Robertson from DR Care Solutions • Helen's GP, Oncologist, Neurologist and any other medical professionals currently engaged in her care. • Financial Advice Team (Kate and Joseph from Lorica Partners) • Estate Planning Team (for review of documents regularly).
Review of Summary for Placements and the date the Family need to determine next steps:	Thursday 17th April 2025

Signature (Consultant):  Date: Thursday 17th April 2025

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